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1.0- Introduction

Vaccinations have long played a crucial role in controlling or eradicating diseases, thereby protecting us from the catastrophic outcomes of unbridled outbreaks and pandemics. The debate on whether vaccines should be mandated has garnered renewed resurgence, especially in the wake of the COVID-19 pandemic. The typical protocol in vaccine development comprises 6 stages before public dissemination; it begins with exploratory research, preclinical trials, clinal trials, regulatory approval processes, mass production, and finally quality control. The clinal trial consists of 3 trials, all of which take years to complete, and are conducted meticulously with watchful eyes (Rao, 2020). However, during the COVID-19 pandemic, it only took one to two years to develop a vaccine; according to John Hopkins, University of Medicine, the 3 phases were conducted concurrently. Due to the time constraints, the vaccines were approved based on data from short-term phase 3 clinical trials, which may have contributed to the populations' distrust towards the efficacy and safety of vaccines despite reassurance from authorities. Additionally, during the pandemic, various governments across the globe – Greece, Austria, and Germany – had attempted to mandate vaccines, albeit receiving pushback from the public, and now has extended beyond COVID.

Before delving into this debate, it is essential to understand a critical term central to this debate, mandatory vaccination. Mandatory vaccination refers to requiring every individual by the law or policy to be vaccinated against diseases that threaten public health, such as the MMR, DTaP, and HPV, with exceptions only for individuals with valid medical reasons such as battling with ailments, receiving medications or treatments that weaken their immune systems, severe allergies towards vaccines or, prior serious reactions from receiving vaccinations.

The discussion on whether vaccines should be mandated has brought about various perspectives and ideas to bolster their respective stances, with arguments centring on maintaining public health, whether the balance between personal autonomy or freedom and the welfare of the collective and the critical role of trust in bodies of power. Mandatory vaccination policies are essential for protecting public health and controlling diseases, but they raise multifaceted ethical and legal questions about individual freedoms and government authority.

2.0 Body

2.1: Maintaining public health

For:

Vaccines have a proven track record of controlling and eradicating diseases with notable examples such as polio or smallpox. An exemplary case is when the World Health Organization, WHO, launched the Global Polio Eradication Initiative in 1988 to curb the effects of polio, saving thousands of children. Since 1988, wild polio cases have been reduced by over 99%; with only 6 reported cases in 2021, a relatively minor number in comparison to an estimated 350,000 cases in more than 125 endemic countries before the initiative. Additionally, the advent of such initiatives has eradicated poliovirus type 2 and type 3 with only type 1 remaining only in Pakistan and Afghanistan as of 2022 (WHO, 2024). Moreover, in 1967, WHO launched a global effort to eradicate smallpox; widespread immunisation and surveillance were conducted globally, which led to the eradication of smallpox in 1980 (WHO, 2022); as a result, smallpox vaccines are no longer provided to the public (*Who Should Get Vaccination*, 2019). Thus, mandating vaccinations is pertinent to eradicate diseases, protecting us from the horrific wrath of infectious diseases.

Against:

While vaccinations are the best means for mankind to eradicate or control viruses, they are not always effective. The elimination of vaccines is a very complex process, and there needs to be keen eyes on factors like mutation rates, animal reservoirs, and differences in the vaccines effectiveness. Just a few mistakes in the production process can also have dire consequences, such as cases of death. For instance, the 1955 "Cutter Incident" occurred when a manufacturing error at Cutter Laboratories caused the production of what was thought to be an inactivated Salk polio vaccine, but it actually contained live poliovirus (Miller, Moro, Cano, & Shimabukuro, 2015). Additionally, every person is different, and the vaccine may or may not be effective in some people and may even result in the death of a person who is sensitive to the vaccine. Some people have a strong initial immune response that wanes over time (Galiza & Heath, 2017).

2.2: Personal autonomy and the welfare of the collective

For:

The concept of herd immunity is one of the main reasons for prioritizing collective welfare over individual autonomy. Herd immunity occurs when a significant portion of the population becomes immune to a disease, thereby reducing the spread of the disease and protecting those who are not immune. This collective immunity is essential to protect vulnerable populations who cannot be vaccinated due to diseases such as allergies or compromised immune systems. With a vaccine mandate, it is ensured that there is a high enough level of immunity in the community to prevent outbreaks of infectious diseases. In order to protect public health, especially that of at-risk populations, it is often necessary to restrict certain individual freedoms for the greater good. Without adequate immunity, preventable diseases may resurface, leading to a public health crisis that could affect millions of people. Thus, the collective benefits derived from herd immunization justify some level of government intervention to mandate vaccination (Fine et al., 2011). This approach is consistent with utilitarian ethics, which focuses on maximizing social well-being.

Against:

While vaccines are indeed one of the most effective tools for protecting individuals against diseases, the act of mandating them may come into tension with others, such as individual liberty and autonomy (i.e., allowing individuals to make their own decisions about their health) (*Public Health - the Nuffield Council on Bioethics*, n.d.). Autonomy is an individual's capacity for self-determination or self-governance, which means that anyone can make their own decisions and choices to perform any actions, essentially allowing them to oversee their own lives of their own volition. Enforcing it sets a dangerous precedent, potentially expanding governmental power on intruding personal health decisions, such as mandating medical procedures or treatments. This action has limited the personal autonomy of individuals, creating a worrying example of the government taking more control over mandating medical procedures and weakening or potentially violating the fundamental human right of individuals – freedom of choice.

2.3: Trust in bodies of power

For:

An important reason for mandatory vaccination is to combat misinformation. Mandatory vaccination policies want to ensure the safety and legitimacy of vaccines. Vaccination mandates are not imposed lightly; they rely on a comprehensive regulatory process that includes rigorous clinical trials, such as those conducted by the U.S. Food and Drug Administration. These trials test safety, efficacy, and potential side effects in different groups. Even after approval, vaccines are closely monitored by systems such as the Vaccine Adverse Event Reporting System (VAERS). Through enforcement, governments and health organizations assure the public that vaccines are safe, effective, and scientifically based. This reduces the spread of misinformation, which can mislead people about the safety of vaccines and lead to lower vaccination rates. Without mandates, misinformation can spread unchecked and jeopardize public health (Wilson & Wiysonge, 2020). Ultimately, mandatory vaccinations help build trust in vaccines by reinforcing their importance and ensuring that the public only receives vaccines that meet strict safety standards.

Against:

Mandatory vaccination may erode trust in government and health authorities, as many view such policies as an overreach, forcing compliance with measures they may not fully understand or agree with. This incomprehension may cause panic and suspicion, especially when there is a lack of transparency about vaccine safety and development. For instance, the 2016 Dengvaxia vaccine incident in the Philippines, where children were given a vaccine later found to increase the risk of severe dengue, led to widespread distrust in vaccines and public health initiatives (Mendoza, Valenzuela, & Dayrit, 2020). This distrust caused a decline in vaccination rates for other diseases, such as measles, resulting in outbreaks (Larson et al., 2018). Instead of fostering trust, mandatory vaccination risks deepening public division. Advocates argue that education, open dialogue, and voluntary programs are more effective in promoting vaccination without undermining public confidence.

3.0 Rebuttal

After a comprehensive review of the common viewpoints, it becomes evident that the argument of eroding trust in governing bodies and health authorities emerges as the weakest argument in the discussion of confidence in bodies of power.

The opposition claims a lack of transparency in vaccine safety and development; however, this is incorrect. Vaccine development follows strict clinical trial protocols, where safety, efficacy, and dosage are assessed rigorously. Additionally, data from these trials are often published in peer-reviewed journals, making the results accessible to the public and the scientific community. Every year, the World Health Organization, WHO, publicly announces the recommended strains of the flu virus that will be included in the vaccine (*Recommendations for Influenza Vaccine Composition*, n.d.), notifying everyone of the vaccine contents. Moreover, the National Pharmaceutical Regulatory Agency requires vaccine manufacturers to provide comprehensive information about the active ingredients, excipients, and other components as part of the registration process (*Drug Registration Guidance Document*, n.d).

To further enhance the opposing argument, the controversial Dengvaxia vaccine incident has been brought up. This incident garnered significant attention due to its public health consequences and the initial lack of clear communication about the vaccine’s risks; however, this was an isolated incident and should not be generalised to encompass all vaccination efforts; it does not invalidate the efficacy or safety of vaccines. While the Dengvaxia incident in the Philippines raised concerns due to inadequate communication, it led to more stringent guidelines and rules that further strengthened the safety and efficiency of vaccines. This demonstrates that the system takes responsibility and learns from its errors, which inadvertently enhances the public’s trust.

It is vital to bear in mind that the discussion of whether vaccines should be mandated should focus on community benefits rather than being driven by a single controversial event twisted to fit a specific narrative. The opposition relies on confirmation bias by highlighting one specific negative case while ignoring the broader, more robust evidence denoting that vaccines, in general, are safe, effective, and have broad public support when properly managed. While it is important to acknowledge such isolated incidents, they should not overshadow the overwhelming evidence of the safety and efficacy of vaccines.

4.0– Conclusion

It is essential to consider multiple perspectives and consult multiple opinions when evaluating the need to mandate vaccination. In terms of eradicating diseases, those in favour argue that vaccines have successfully protected us from the wrath of the unbridled spread of infectious diseases while those opposing the mandate claim that reality is often disappointing; eradicating diseases is no small feat, often requiring tremendous near-perfect decisions and can be crumbled by a single mistake. The balance between personal autonomy and the welfare of the collective is also brought to light. Those in favour of mandating vaccination argue that we should consider the collective good and help those who are vulnerable by achieving herd immunity. In contrast, those opposing the mandate of vaccines claim that mandating vaccines violates human rights and liberty. Finally, regarding the trust in bodies of power, the supporting stance claims that this is not an issue if the government and health authorities maintain transparency, clear communications and provide valid scientific evidence while those challenging the mandate argue that such policies may be viewed as overreach by many, leading to doubts regarding the needs of such policies.

Based on the aforementioned arguments, the proposition’s viewpoint on this issue leans towards the collective good and the future of humanity while stressing the need for meticulous planning while the opposition raises valid concerns about individuals’ rights and freedoms as well as the fragility of such policies. It cannot be denied that mandatory vaccination can significantly reduce the spread of infectious diseases such as COVID-19, protect vulnerable populations, and achieve herd immunity; however, we cannot ignore the voices of the masses; we must not ignore their rights and freedom to personal choice and maintain their trusts in their respective governments and health authorities. The key to addressing this conundrum lies in the trust between the public and health authorities, ensuring that individuals are well-informed about the benefits of vaccines while respecting their freedoms and autonomy, and creating a society that aims to promote public health without compromising individual rights. In conclusion, we are in favour of mandating vaccinations.

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